स्वास्थ्य एवं
परिवार कल्याण मंत्रालय
MINISTRY OF
HEALTH AND FAMILY WELFARE

सत्यमेव जयते

NATIONAL HEALTH MISSION
राष्ट्रीय स्वास्थ्य मिशन

National Health Mission
NPCC Meeting

Arunachal Pradesh
1st April, 2026

Table of Content

Review & Performance

- ✓ NHM Finance
- ✓ PM-ABHIM & FC-XV
- ✓ SDGs & Disease Elimination
- ✓ HRH
- ✓ Drugs & Diagnostics
- ✓ IPHS Compliance
- ✓ Key Deliverables
- ✓ CRM ATR

Annexures

- ✓ Capacity building
- ✓ ASHA incentives
- ✓ IT Interventions & systems
- ✓ IEC & Printing
- ✓ State specific Initiatives & Innovations
- ✓ Program Management activities
- ✓ Revalidation

PD Financial Proposals FY 2026-27

- ✓ HSS (Urban)
- ✓ HSS (Rural)
- ✓ NCD
- ✓ NDCP
- ✓ RCH

AS/ JS- Programme divisions

Dr. Rakesh Gupta, Additional Secretary	Ms. Meera Srivastava	Shri Sibin C	Shri Nikhil Gajraj	Shri Pushpendra Rajput	Shri Lalit Wadhwa	Shri Vijay Nehra
IDSP	RMNCHA+	HSS-Urban	TB Mukt Bharat Abhiyan	NPNCD	NPCBVI	NMHP
NPCCHH		HSS-Rural	NVBDCP	NPPC	NPHCE	Tele-manas
NRCP/ PPCL/ Snake Bite		IT	NSCAEM and Blood Cell	PMNDP	NOHP	
		HMIS	NVHCP		NIDDCP	
		MMP Cell	NTCP		NPPCD	
			NLEP			

Program Division PIP Proposals

RCH

- Maternal Health
- JSY
- CAC
- PCPNDT
- Child Health
- Immunization
- Adolescent Health
- Family Planning
- Nutrition

NDCP

- NVHCP
- TB Mukt Bharat Abhiyan
- NLEP
- NCVBDCP
- IDSP
- **NSCAEM and Blood Cell**

NCD

- NP-NCD
- PMNDP
- NPPC
- NMHP
- NPHCE
- NPCBVI
- NPPCD
- NOHP
- NIDDCP
- NTCP
- NPCCHH

HSS

- HSS- Urban
- HSS-Rural

PM-ABHIM & FC-XV

Arunachal Pradesh – PM-ABHIM Snapshot

Physical Progress (FY 2021-22 to FY 2025-26)

Components	Total Units Approved	Work Started	% Work Started	Work Completed	% Work Completed	Functional
District Integrated Public Health Labs (IPHL)	22	11	50%	5	23%	5
Critical Care Blocks (CCB)	1	0	0	0	0	0

Arunachal Pradesh – XV-FC Financial Progress

(Rs. in Crore)

Components	2021-22			2022-23			2023-24			2024-25			2025-26		
	Approval	Release	% Exp.	Approval	Release	% Exp.	Approval	Release	% Exp.	Approval	Release	% Exp.	Approval	Release	% Exp.
FR-1: Building-less SHC, PHC & CHC	1.04	1.04	100.00 %	1.06	1.06	0.00%	16.26	16.26		20.07	-		22.39	-	
FR-2: BPHU	22.94	22.94	74.65%	22.64	22.64	0.00%	14.44	14.44		12.23	-		11.61	-	
FR-3: Support for Diagnostic (Rural)	9.78	9.78	100.00 %	9.80	9.80	100.00 %	10.29	10.29	0.00%	12.31	12.31		13.05	13.05	
FR-4: Conversion of Rural SHC & PHC to AAM	6.65	6.65	100.00 %	6.65	6.65	0.00%	1.11	1.11		1.12	-		1.12	2.00	
FU-1: Support for Diagnostic (Urban)	1.30	1.30	100.00 %	3.07	3.07	100.00 %	3.90	3.90	0.00%	2.48	2.48		2.60	-	
FU-2: U-AAM	5.24	5.24	100.00 %	5.24	5.24	0.00%	3.71	3.71		5.57	-		5.69	-	
TOTAL	46.94	46.94	87.62%	48.45	48.45	84.89%	49.71	49.71	0.00%	53.78	14.79		56.46	15.05	

% Expenditure (as on Marchec-2026) is against the amount released

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SDG and Disease Elimination

SDGs and Disease Elimination

S No	SDG Indicator	Current Status of State / UT	SDG Target
1.	3.1.1 Maternal Mortality Ratio	N/A	Less than 70
Total No. of Districts in the State/UT		**Total number of Maternal Deaths (MDs) in the State 2024-25**	**Major Cause Reported in 2024-25**
28		HMIS - 3	**HMIS-** Others/Unknown – 64.3% (2); Hypertensive Disorder in Pregnancy – 5.51% (1)

Districts contributing most no. of MDs out of total reported MDs in 2024-25	East Siang (2); Namsai (1)
Place of Deaths	Facility deaths (100%)

State/UT's Action Plan	MoH&FW Comments
Interventions proposed on the basis of causes of MDs	1. Only 7 FRUs are functional out of 15 designated in the state. 2. % ANC registered 1st Trimester is only 62.2% in FY 2024-25. 3. % PW receive 4 ANC Check ups is only 69.9% in FY 2024-25. 4. **SUMAN notification-** State has notified only 29.6% (181/ 620 facilities). 5. **PMSMA-** Only 5.53% of High risk pregnancies have been identified since inception.

Child Health – Arunachal Pradesh

S No	SDG Indicator	Current Status of State / UT	Current Status at National Level	SDG Target by 2030
1.	Infant Mortality Rate (IMR) – **SRS 2023**	20	25	-
2	Stillbirth Rate (SBR) **HMIS 2025-26**	8.8	9.5	-

Total Districts - 28 | **No. of SNCU/ NICUs - 5** | **No. of DEICs - 3**

	Districts with high Mortality	Main Reason of Mortality
SNCU Mortality (**FBNC Portal,** Apr-Dec, 2025) – **27 Neonatal Deaths**	East Siang (5.6%), Papumpare (4.0%)	**SNCU/ NICU Mortality (27):** Respiratory Distress Syndrome (18.5%), Birth Asphyxia (14.8%), Meconium Aspiration Syndrome (7.4%), ***Any Other (55.6%)*** – **FBNC Portal** **Infant Mortality (52):** Prematurity (23.1%), Asphyxia (11.5%), Sepsis (5.8%) ***Other (57.7%)*** – **HMIS Portal**
Infant Mortality Rate (**HMIS**, Apr- Dec, 2025)	Longding (13.8), West Siang (8.1), Namsai (6.6), Lohit (5.6)	

	HR Availability	Equipment Availability	Bottom 30% Districts
RBSK – Functionality of DEIC	**Overall 52% HR available**	**Overall 67% Equipment available**	**Pashighat 74%** **Tezu 25%**

Action Plan as shared by State	MoHFW Comments
• Revalidation amount of 87.46 lakhs has been proposed for equipment required for 3 functional DEIC Tezu, Pasighat and Longding. The proposed amount was approved for the FY 2025-26 but couldn't be executed therefore it has been proposed under Revalidation. • 37% of total in position recruitment has been done in three functional DEIC but due to lack of applicant despite frequent advertisement , 63% of HR is left unfilled.	• State need to strengthen reporting under MPCDSR Portal [**0 Infant Death** reported in MPCDSR v/s 52 Infant Deaths in HMIS]. It will also help in categorization of ***Any Other deaths.*** ❖ 17 Districts reported ***Zero death*** in HMIS for the period of April- December, 2025. • **State to expedite the functionality of the DEIC at Papum Pare** • HBNC home visit coverage need improvement (<90%). Poor visits in; Dibang Vally (13%), East Siang (36%), Pakhe Kesang (40%), Capital Complex (43%), East Kemang (46%) (25-26) against State average 83%. • HBNC: Poor community referral (11%) of identified sick newborn. Out of 23 districts, 20 districts have no identification of sick newborn, so no referral (25-26).

SDGs and Disease Elimination

S No	SDG Indicator	Current Status of State / State	SDG Target
1.	3.7.1 Proportion of currently married women of reproductive age (15–49 years) who have their need for family planning satisfied with modern methods	**60.2 %**	74.2 % (National Average)

State/State's Action Plan	MoH&FW Comments
	• The State has Total unmet need of **12.5 %.** (India – 9.4%) • **Modern contraceptive use** is 47.2% in NFHS-5 (India-56.5). • State to continue promoting "Heathy Timing and Spacing of Pregnancies for the wellbeing of mother and child". • State to focus Post Pregnancy Family Planning. • Promote Expanded Contraceptive Basket of Choice. • Strengthen counselling services to improve quality of care in family planning service delivery. • State to achieve 100% Operationalization of FPLMIS (upto the level of Sub centre). At present State is at 46.53 %.

SDGs and Disease Elimination

S No	Disease Elimination targets	Current Status	Target
1	TB Mukt Bharat Abhiyan aims to achieve SDGs and eliminate tuberculosis (TB).	**Incidence – 191** **Mortality - 18**	➢ **47 / lakh** population (80% reduction in TB incidence) ➢ **3 / lakh** population (90% reduction in TB mortality)

State	Indicator	Achievement			Factors affecting
Arunachal Pradesh	Notification Vs Estimates	**>90%**	**50-90%**	**<50%**	Undernutrition (35%), High tribal population (69%), Migrant population (construction workers)
		3 districts	**5 districts**	**7 districts**	
	Death Rate	**>5%**	**3-5%**	**<3%**	Undernutrition (51%)
		4 districts	**7 districts**	**4 districts**	

State/UT's Action Plan	MoH&FW Comments
• Compilation of Vulnerable Population forecast requirement of Health Workers, NAAT Facilities, X- ray etc. • Training on campaign operation, micro plan development & reporting for relevant Health care worker. • The campaign's financial needs will be met through existing resources under NTEP. Any resource gaps will be addressed through supplementary PIP Funding. • Regular Internal Evaluation and Review Meeting with Districts.	• State needs to focus on 100% screening of vulnerable population through X-ray • Scale up TPT beyond household contacts to cover other risk groups • Saturate differentiated TB care & nutrition interventions

Progress under TB Mukt Bharat Abhiyan (07.12.24 – 14.02.26)

- Vulnerable population screened – **16%** (37,277 / 2.40 lakhs)
- X-ray examination rate – **19%** (7,050 / 37,277)
- NAAT % of all tests – **95%** (16,487 / 17,428)
- Ni-kshay Poshan Yojana (all benefits paid) – **10%** (337 / 3225)- IFMIS integration in process – External Beneficiary Paid- 269, Amount Paid- 0.18 cr
- Ni-kshay Mitra (at least one food basket given) – **29%** (868 / 3,013)
- Differentiated TB Care (% assessed) – **30%** (1,007 / 3,377)
- TPT initiated – **23%** (302 / 1,332)

- NAAT – 39 machines (13 CBNAAT & 26 Truenat) – 15% Utilization in 1 shift
- Handheld X-ray – **61** (15 available + 46 in pipeline)

Ni-kshay Poshan Yojana			
Year	**Eligible Beneficiaries (ex. forgone)**	**Beneficiaries Paid All benefits**	**Beneficiaries to be Paid All benefits**
2024	3169	1508	1661
2025	2779	13	2766

SDGs and Disease Elimination

S No	Disease Elimination targets	Current Status	Target
1	Leprosy: National roadmap (NSP 2023-27) aims at interruption of transmission at district level by 2027).	Total districts- 25 Districts Achieved IOT- 13	Interruption of transmission at district level

State/UT Name	Above 90% *(Illustrative)*	80 – 90% *(Illustrative)*	70 – 80% *(Illustrative)*	60 – 70% *(Illustrative)*	50 – 60% *(Illustrative)*	Less than 50% *(Illustrative)*
No. of Districts achieved IOT-13					Changlang, Dibang Valley , Dibang Valley Lower, Kameng West , Kamle , Lohit , Longding, Lower Siang , Namasai , Siang East , Siang West , Subansiri Upper, Tirap	

Data of Arunachal (2024-25) & (2025-26)

F.Y.	Total New Cases	Total G2D Cases	Total Child Case
2024-25	26	1	1
2025-26	24	0	0

State/UT's Action Plan	MoH&FW Comments
	• **13 districts have achieved IOT status** i.e Changlang, Dibang Valley , Dibang Valley Lower, Kameng West , Kamle , Lohit, Longding, Lower Siang , Namasai , Siang East , Siang West , Subansiri Upper, Tirap • **12 districts have achieved Elimination Status** i.e Anjaw, Kameng East , Kradaadi , Kurung Kumey , Lepa Rada, Pakke Kessang, Shiyomi, Siang , Siang Upper , Subansiri Lower , Subansiri Upper, Tawang. • **State to declare leprosy as a notifiable disease.** • Strengthen SDR-PED Coverage, Ensure 100% PEP coverage. 02 contacts per index case. • Sustained Surveillance in the districts achieved IOT. • Implement focused, district-specific plans covering hard-to-reach and vulnerable areas to achieve interruption of leprosy transmission by 2027. • Strengthen RBSK and RSSK for screening of leprosy and referral of suspects to the nearest health facility. • Intensify IEC activities across the state. • Sensitize private practitioners to report leprosy cases to the concerned District Leprosy Officer (DLO) • Strengthen ASHA training, with emphasis on early signs such as weakness of limbs and difficulty in holding objects or walking. • Nikusth entries are 14 % and need to verify from MPR and ensure 100% entries and analysis

SDGs and Disease Elimination

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Malaria : Disease status

	2021	2022	2023	2024	2025
Cases	**8 (Pv-6, Pf-2) Ind 07, Imp 01**	**18 (Pv-15, Pf-3) Ind 13, Imp 05**	**11 (Pv-6, Pf-5) Ind 05, Imp 06**	**19 (Pv-17, Pf-2) Ind 10, Imp 09**	**32 (Pv-21, Pf-11) Ind 17, Imp 15**
Deaths	**0**	**0**	**0**	**0**	**0**

- **State falls under category-1; API (0.02) in 2025 and all districts have API<1** and **ABER is 10.76.**
- 16 districts are eligible for Sub-National Malaria Elimination Verification (2022-24) Longding, Kurung Kumey, Kra Daadi, Lower Subansiri, Kamle, Pakke Kessang, East Siang, Lower Dibang Valley, East Kameng, Tawang, Dibang Valley, Upper subansiri, Anjaw, West Kameng, Leprada and Siang. State has submitted **16 draft dossier**
- **5 more districts Lower Siang, Shi Yomi, Upper Siang, West Siang and Namsai reported zero indigenous cases (2023-2025)**.
- State has two functional entomological zone Kimin and Miao (Miao (East zone) regularly shares the reports)
- Treatment completion and treatment outcome data entered for **93.8%** cases on **IHIP portal**
- Fund utilisation in 2025-26 (Y2) **under GFATM 63.5% (till February), & Under DBS 30% (till January)**
- Stock data is not updated on **DVDMS.**
- Cross checking of slides should be done as per guidelines **(475 sent to ROHFW against target of approx. 859)**

SDGs and Disease Elimination

Disease Elimination targets	Current Status	Target
National Framework for Malaria Elimination in India 2016-2030, aims to eliminate malaria by 2030	32 (Pv-21, Pf-11) (Imported=14; Indigenous=18)	Zero indigenous cases by 2027

State/UT's Action Plan	MoH&FW Comments
· IEC/BCC activities will be intensified to promote early fever reporting, complete treatment, and regular use of LLINs. · 100% testing of all fever cases will be ensured through strengthened active and passive surveillance with adequate availability of RDTs and anti-malarial drugs. · Focused interventions in endemic districts, especially Tirap and Changlang, including micro-stratification, intensified surveillance and LLIN coverage. · Papum Pare, Lohit and reporting a higher number of non-indigenous cases 2024 -25 will be prioritized for intensified fever testing, strengthened surveillance, and source reduction/vector control activities to prevent local transmission. · Screening and Monitoring of migrant and mobile populations · All malaria cases and fever sample testing data will be updated regularly in the IHIP VBD portal to ensure timely reporting, monitoring, and data-driven action.	• Sustain "Zero indigenous cases" in districts eligible for SNV • Active surveillance should be strengthened and treatment completion should be ensured • Proper epidemiological investigation should be undertaken before classifying cases into indigenous and imported • State to ensure Cross notification of imported cases to native states • Focal IRS may be undertaken in vicinity of positive cases if supported by vector surveillance • Enhance surveillance activities in bordering and hard to reach areas • State to ensure adequate utilization of funds under DBS and GFATM budget

SDGs for NCDs

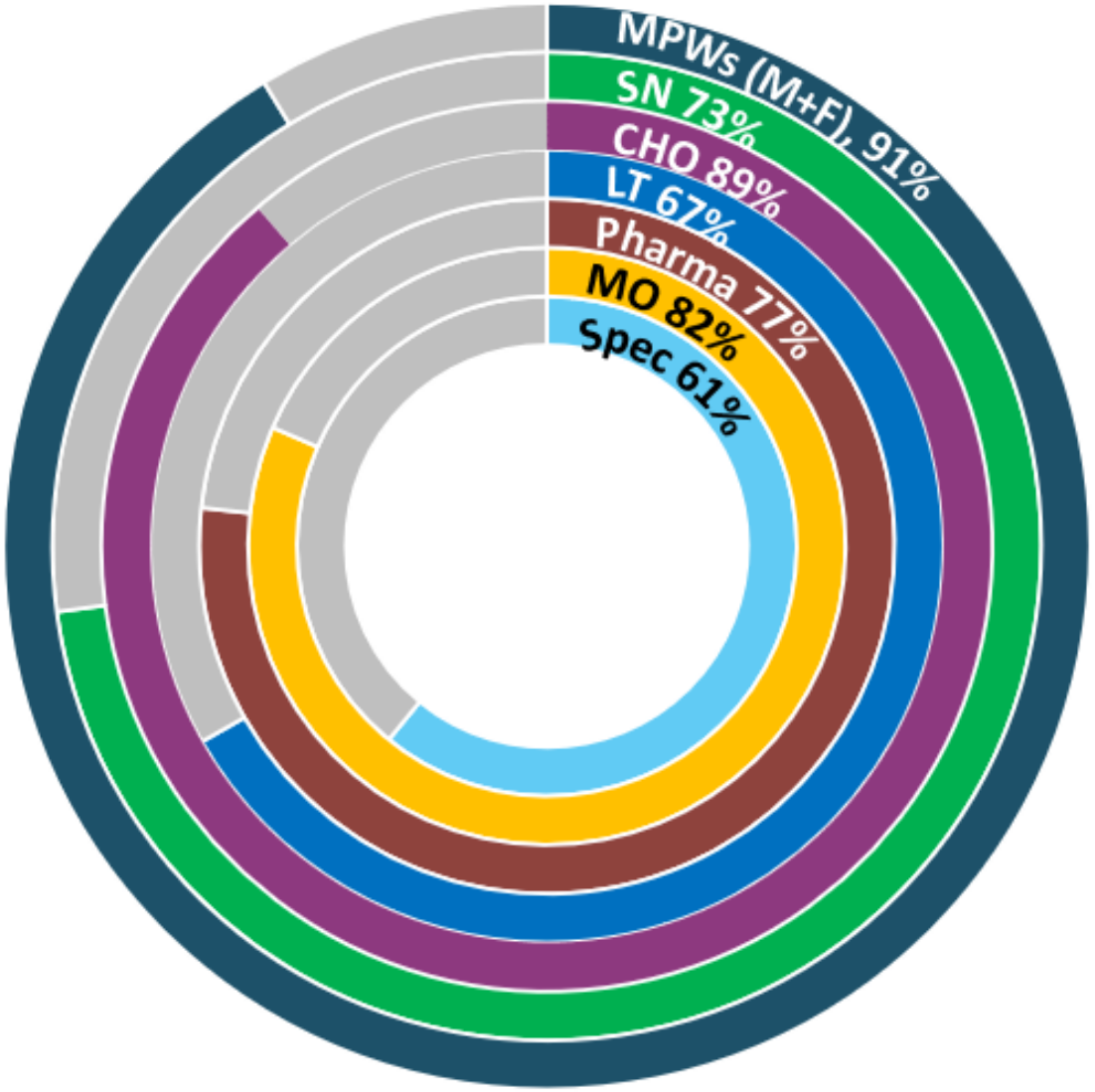
S No	SDG Target
1.	3.4 Reduce by one third premature mortality from non-communicable diseases through prevention and treatment and promote mental health and well-being

Indicator 3.4.1: Mortality rate attributed to cardiovascular disease, cancer, diabetes or chronic respiratory disease

State/UT's Action Plan	MoH&FW Comments
	1. State needs to make specific proposals for all 5 existing and five new diseases. 2. Make full use of funds allocated for NCDs. 3. Plan for enhanced interventions in NCDs.

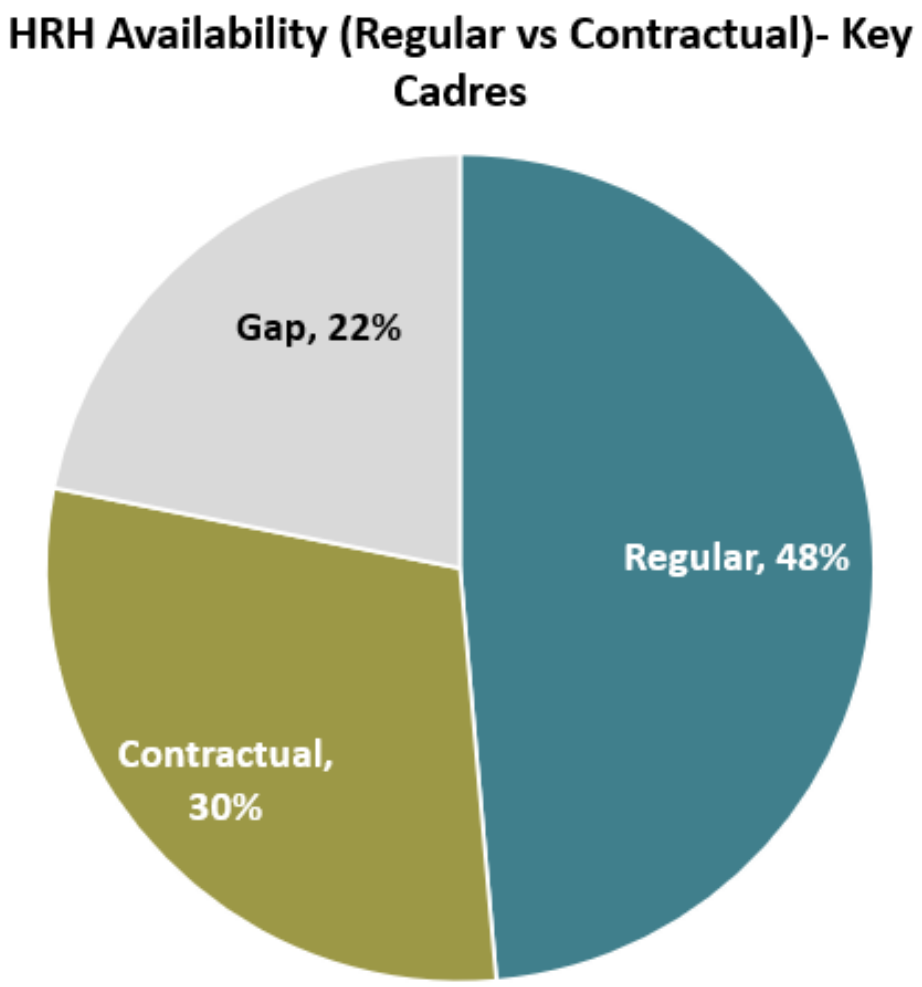
HRH

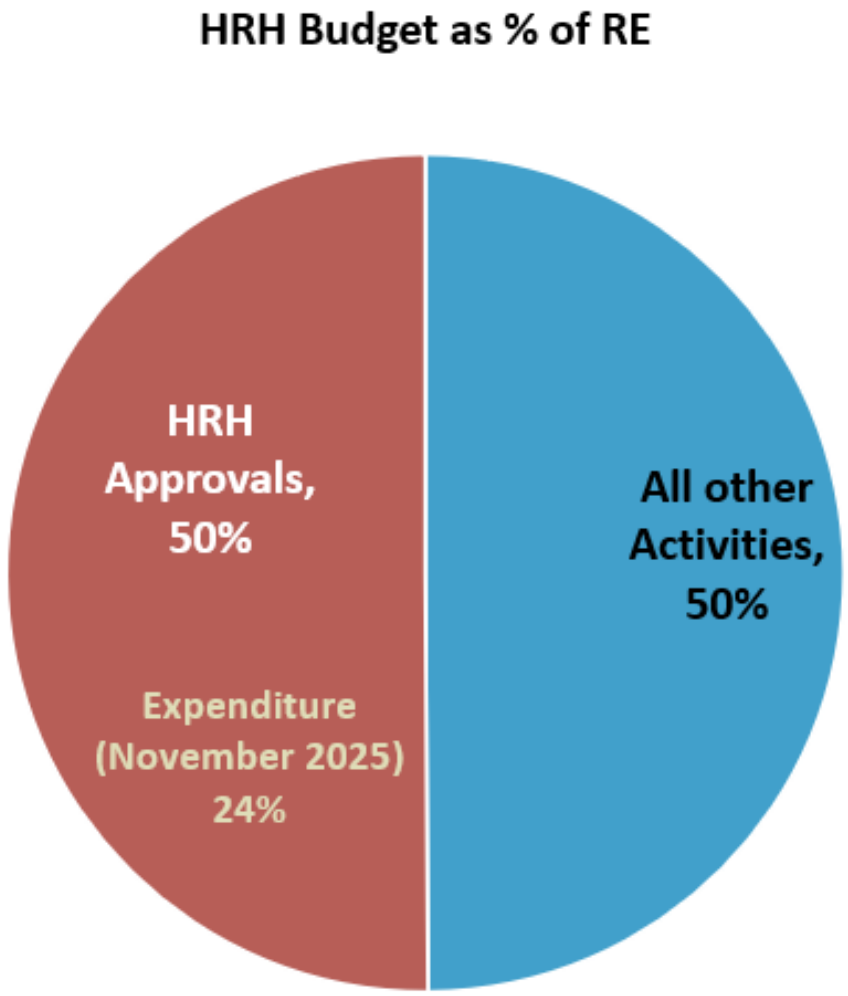
HRH Availability- Key Cadres



Disclaimer: The data in this presentation is intended only for the purpose of NPCC discussion. It should not be used independently for any other analysis or interpretation.

HRH Availability and Budget Share





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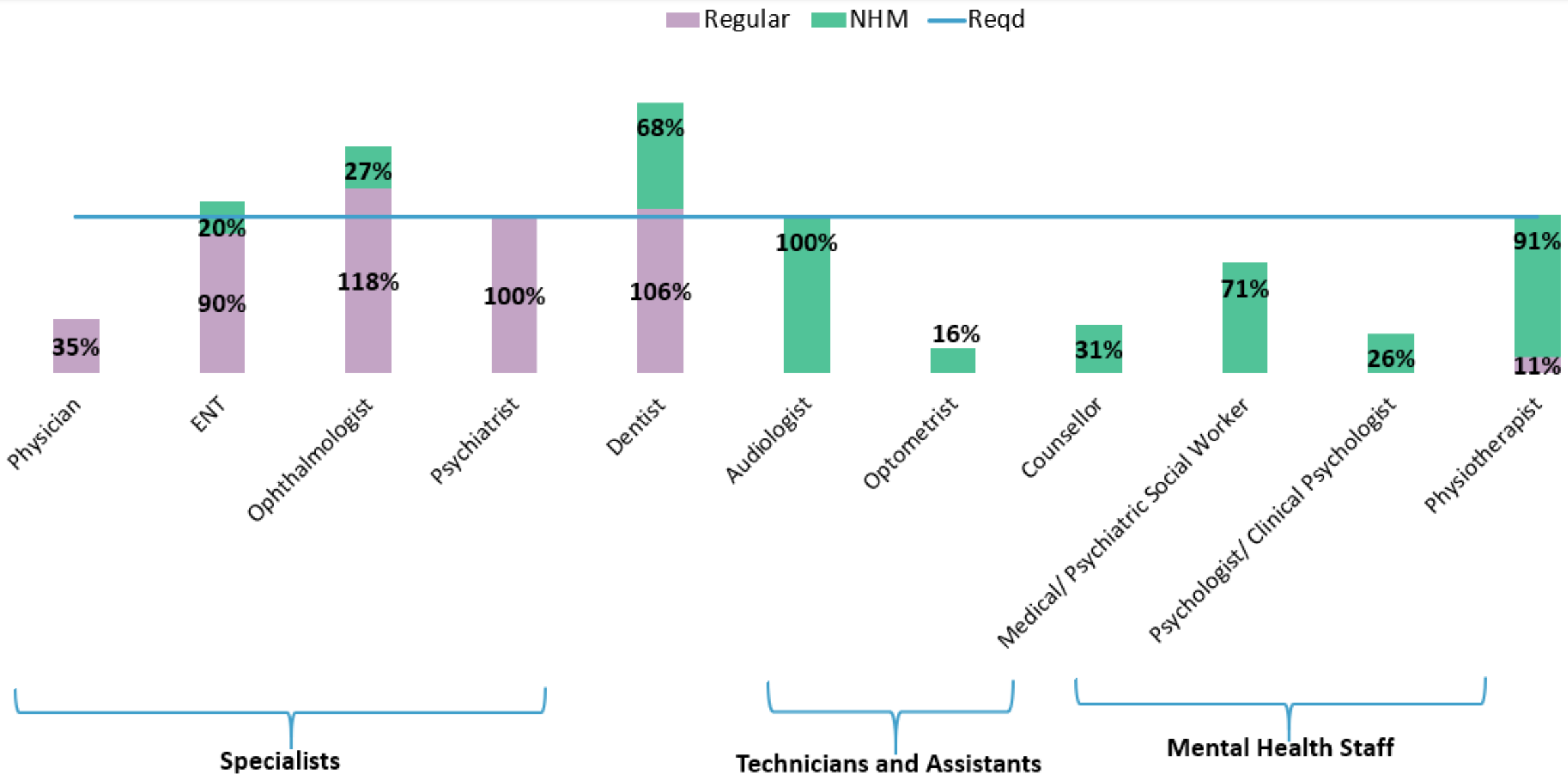
Important Cadres for NCD



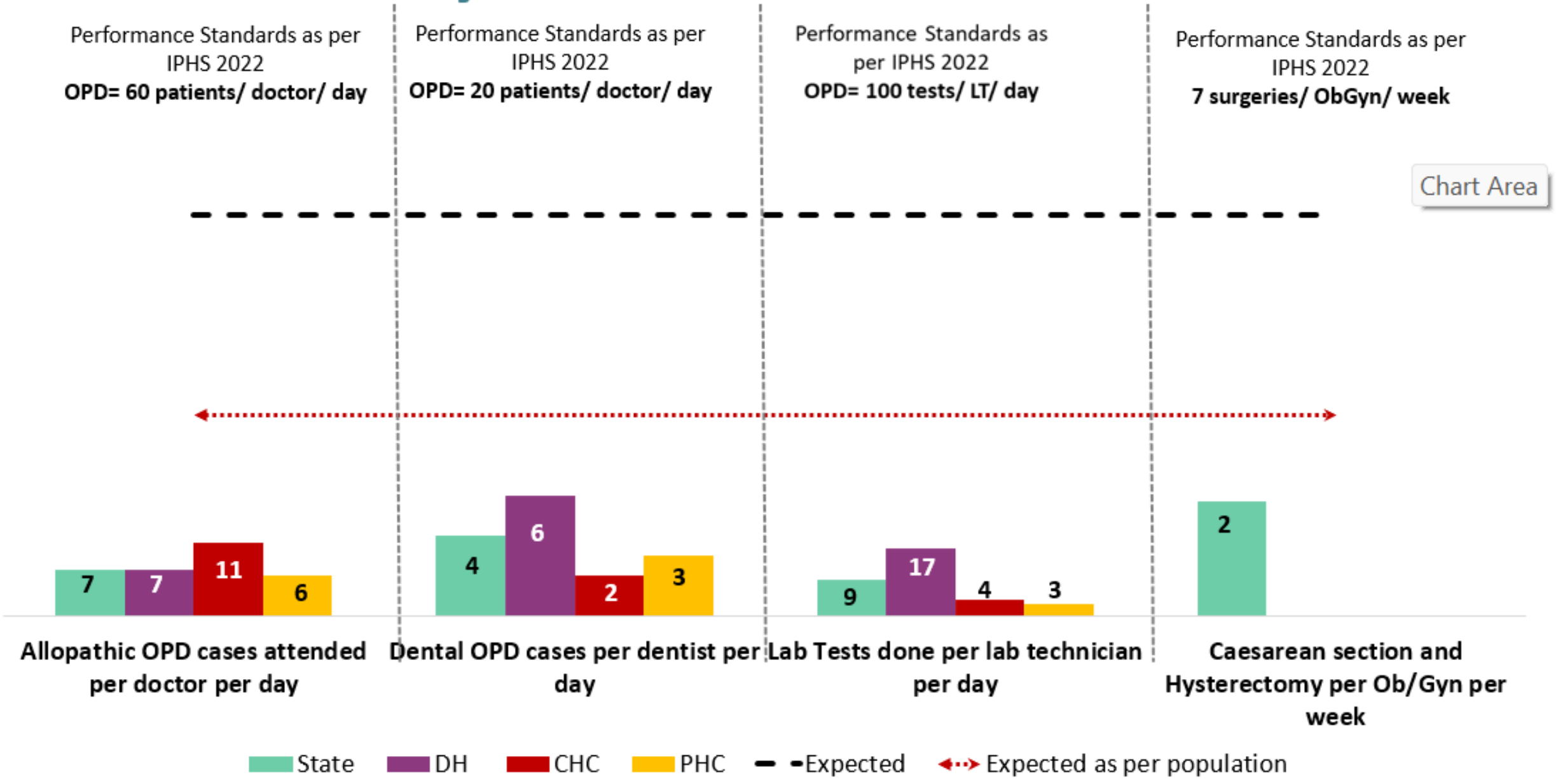
Important Cadres for NCD- Programme Management

Level of PMU	Consultant	Coordinator
State	State Consultant NTCP – (1) Consultant NOHP and NPPCD – (1) Consultant NPCCHH – (1)	NCD State Program Coordinator – (1) Program Assistant NTCP and NPPCD – (2)
District	District Consultant NTCP- (13)	NCD District Program Coordinator – (20)

() indicates approved posts

*Additional posts of State Fin cum Logistics Consultant (1), State Finance Consultant (1), District Monitoring and Evaluation Officer (2) and District Accounts Staff (19) are present at State and District level.

HRH Productivity



Key Deliverable 2025-26 Achievement

HRH Key Cadres	Requirement	Regular		Contractual (NHM+NUHM+PM-ABHIM)		Total (Regular + Contractual)		Achievement 2025-26	Target 2025-26 as per RoP
		Sanctioned	In Place	Approved	In Place	Total Sanctioned	Total In-Place	% of HRH Available	% of HRH Available
MPW (F+M)	964	1327	678	350	202	**1677**	**880**	91%	70%
Staff Nurse	1910	908	766	1150	626	**2058**	**1392**	73%	55%
CHO	347			317	308	**317**	**308**	89%	-
Lab Technicians	530	320	109	245	245	**565**	**354**	67%	55%
Pharmacists	297	240	228	1	0	**241**	**228**	77%	85%
Medical Officers (MBBS)	638	557	467	56	53	**613**	**520**	82%	85%
Clinical Specialists	266	251	144	32	18	**283**	**162**	61%	50%

Drugs & Diagnostics

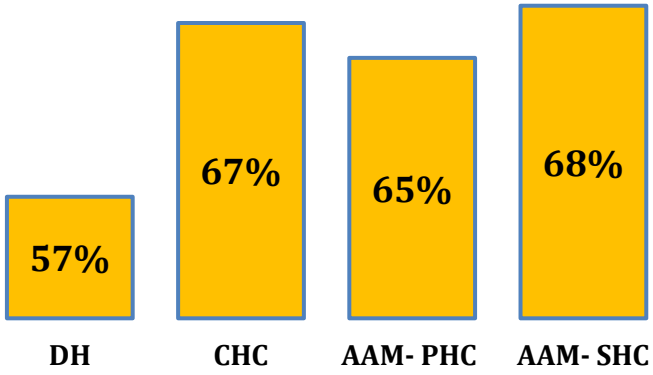
Health Systems Strengthening (HSS)
Free Drugs Service Initiative

Free Drug Service Initiative (FDSI)

Level of facilities	IPHS EML	State Notified EDL	State EDL (Molecules)	Medicines common between IPHS and State EDL (Molecule)	Stock Availability as reported by the State
District Hospital	381	459	298	216	16.12%
Community Health Centre	300	459	302	202	12.20%
AAM – PHC	172	179	123	112	24%
AAM - SHC	106	102	75	72	21.56%

Health Systems Strengthening (HSS)
Free Drugs Service Initiative: State Attributes

IPHS EML Compliance %



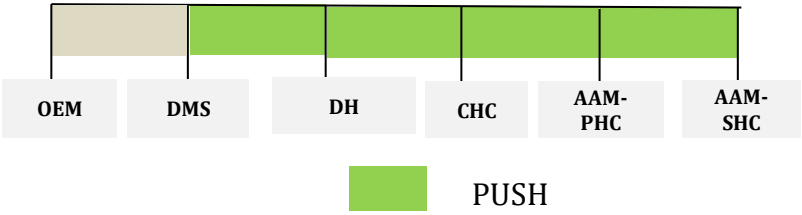
IT Enabled System

DVDMS
Implementation Status

DH	CHC	AAM PHC	AAM SHC

Integrated with the DVDMS Central Dashboard

Transportation Mechanism



EDL Notified : 2024

Total warehouse - **30**

Active RCs State Reported - 86
DVDMS - 5

Challenges

1. Stock availability of medicines on DVDMS Central Dashboard is less than 50%.
2. There is no centralized procurement system in the State.

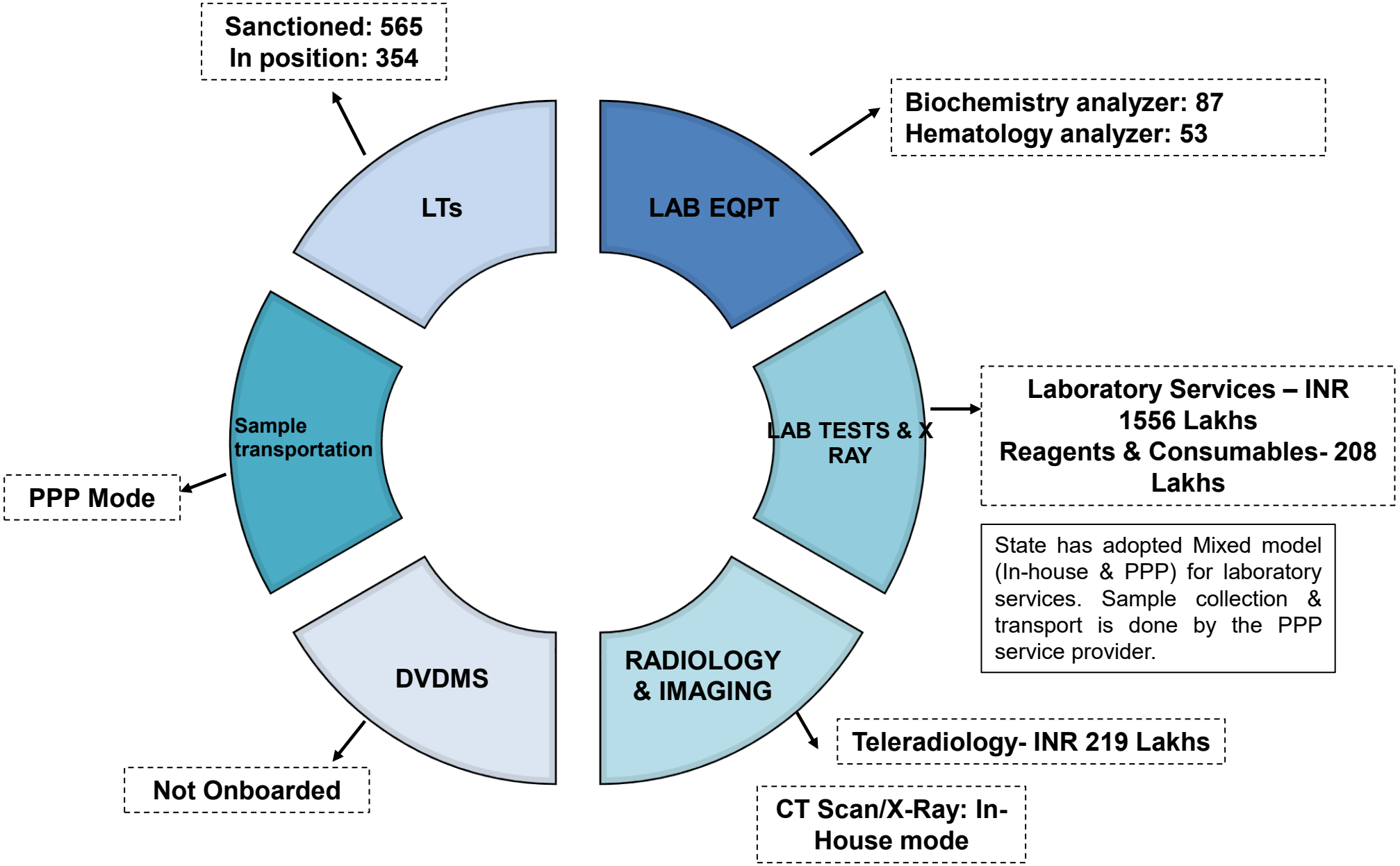
FDSI

Level of facilities	No. of Diagnostic Tests as per FDSI	No. of Diagnostic Tests as per ODK report Jan 2025	No. of Diagnostic Tests as per ODK Report Jan 2026	Gap
AAM-SHC	14	5	7	**7**
AAM-PHC	63	8	18	**45**
Community Health Centre	97	13	30	**67**
District Hospital	134	55	68	**66**

AVERAGE PATIENT TO TEST RATIO				
Type of Health facility	AAM-PHC	CHC	DH	State Average
Average Patient test ratio	1	1	2	1

*** National Average patient to test ratio: 3**

ARUNACHAL PRADESH-FDSI



NQAS

National Quality Assurance Standards Status

Type of Healthcare Facility	Total no. of facilities (as per HDI 2022-23)	No. of NQAS State certified facilities	No. of NQAS National certified facilities	Total certified (State + National)	Percentage
DH	20	1	0	1	5%
SDH	0	0	0	0	nil
CHC	57	0	0	0	0%
AAM PHC	127	1	1	2	2%
AAM UPHC	6	0	0	0	0%
AAM SHC	289	13	10	23	8%
Total	**499**	**15**	**11**	**26**	**5.21%**

Recertification Due: 0

Quality Cadre (filled)	2/ 2 State	0 / 0 District
Last SQAC conducted	February 2026	
IA Training in FY 2025-26	1 batch (Aug 2023), Refresher training of existing Internal assessor (May 2025)	

Prioritization of Facilities for NQAS Certification

Internal assessment	87/ 23 facilities (scored >70% in IA)	
LaQshya (certified)	4 Labour Room	3 Maternity OT
MusQan (certified)	nil	
Kayakalp (Kayakalp compliant in FY 2024-25, not NQAS certified)	25	

Total Quality trained professionals in the State							
External Assessors	19	Internal Assessors	99	Ayushman Assessors	0	TISS candidates	0

IPHS Compliance

IPHS Compliance Percentage Across Facilities- State Summary

Source: IPHS Dashboard 20th March 2026

State	Facility Type	Number of Facilities (as per HDI 2022-23)	IPHS Compliance Score						% Assessed
			Aspirant		Progressive		Compliant	Total	
			Less than 25%	25%-50%	50%-69%	70%-79%	More than 80%		
Arunachal Pradesh	DH	20	5	15	1		1	22	110
	SDH	0						0	
	CHC	57	19	35	2			56	98
	PHC	133	26	83	22			131	98
	SHC	401	24	156	162	49	9	400	100
Arunachal Pradesh Total		611	74 (12%)	289 (47%)	187 (30%)	49 (8%)	10 (2%)	609	100

IPHS-compliant facilities have remained same with **2% compliance** (1 DH & 9 SHC)

*As per HDI 2022-23, there is no **shortfall** of health facilities in rural areas*

IPHS Compliance Percentage Across Domains- State Summary

Source: IPHS Dashboard 6th March 2026

Domains	Aspirant		Progressive		Compliant	Total
	Less than 25%	25%-50%	50%-69%	70%-79%	More than 80%	
Diagnostic	312 (50%)	111 (18%)	97 (15.9%)	28 (4.6%)	61 (10%)	609
Drugs	126 (20.6%)	350 (57.3%)	105 (17.2%)	18 (3%)	10(1.6%)	
Human Resource	26 (4.25%)	124 (20.3%)	111 (18%)	78 (12.8%)	270 (44%)	
Infrastructure	96 (15.7%)	170 (27.9%)	273 (44.7%)	50 (8.2%)	20 (3.3%)	
Governance	212 (34.7%)	129 (21.11%)	176 (26.8%)	28 (4.6%)	64 (10.5%)	
Services	190 (31%)	155 (25.4%)	112 (18.33%)	59 (9.7%)	93 (15.2%)	

In comparison to the status in **MTR March 2025**, the percentage of facilities achieving IPHS compliance (>80%) has remained same with a minor increase in the compliance percentage of Diagnostics, Governance, & Services.

स्वास्थ्य एवं
परिवार कल्याण मंत्रालय
MINISTRY OF
HEALTH AND
FAMILY WELFARE

सत्यमेव जयते

NATIONAL HEALTH MISSION
राष्ट्रीय स्वास्थ्य मिशन

Key Deliverables- (FY 2025-26)

KD's Performance (2025 – 26 up to December 2025)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement
1	Output	Maternal Health	ANC Coverage	Percentage	95%	**95.7%**
2			ANC registration in 1st trimester of pregnancy (within 12 weeks)		75%	**62.7%**
3			Pregnant Women who received 4 or more ANC check-ups		80%	**68.3%**
4			Identification of HRP		10%	**10%**
5			Management of HRP		100%	**56%**
6			Institutional Deliveries		90%	**94.1 to total rep deliv. 70.5 to ANC regist.**
7			National Certification of LRs & OTs under LaQshya		LR-5 OT-4	**LR-3 OT 2**
8			Public Health facilities notified under SUMAN		78	**62**
9			Maternal Death review mechanism		80%	**100% (1 death reported)**
10			NQAS certification for SUMAN notified Facilities	Number	40	**10**

Janani Suraksha Yojana (JSY)
KD's Performance (2025–26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement
1	Output	RCH/ MH	Number of JSY Beneficiaries	Numbers	100% of the targets approved in the RoP	**66% (Up to Q3)** **58% (against full FY targets)** • Achievement (Up to Q3) :**11,840** • RoP Target: (Numbers) : **23,830**

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement
1	Output	Comprehensive Abortion Care	**RoP Key Deliverables** Public Health Facilities equipped with Drugs (MMA Combi pack/ Mifepristone & Misoprostol), Equipment (MVA/EVA) and Trained Provider (MTP Trained MO/OBGYN) for providing CAC services against the total number of Public Health Facilities as per RoP targets (Source: State Reports)	Percentage	120 Facilities • MCH: 1 • DH including women & Children Hospital : 25 • SDHs :0 • CHCs (FRUs) & Other Sub District Level Hospitals: 1 • 24 x 7 PHCs, Non FRU CHCs: 93	14% (17 Facilities) (Source: CAC 3rd Quarterly Report)
2	Output	Comprehensive Abortion Care	% of Medical Officer trained in CAC against the RoP approval .	Percentage	26 MBBS Doctors	0% (Source: CAC 3rd Quarterly Report)

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target FY 2025-26 (till September 2025)	Achievement FY 2025-26 (till September 2025)**
17	Output	PCPNDT	Total Number of meetings conducted by District Advisory Committees (DAC) in the state/ UT Numerator- Total No. of meetings actually conducted by all districts in the state Denominator- No of district *6 **(6 meetings / Year/ District)*** **(Target 100%= 150 meetings per year in 25 districts)**	Percentage	**75 (100%)**	**0 (0%)**

* As per Rule 15 of the PC&PNDT (Prohibition of Sex Selection) Rules, 1996

** Source: State Quarterly Progress Report

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Indicator Statement	Unit	Target	Achievement
12	Output	SNCU successful discharge rate out of total admission (%)	Percentage	Sustain > 80%	89.2%
13	Output	HR training in Newborn Health - Percentage of Pediatrician / Medical Officers and Staff Nurses trained in FBNC and NBSU Training Package	Percentage	100% (5 Batches = NSSK: 3 + NBSU: 2)	100% (5 Batches = NSSK: 3 + NBSU: 2)
14	Output	Percentage of Child Death Reported against Estimated deaths (Total Infant Deaths = 124)	Percentage	70% (294 Infant Deaths) **Estimated 220 Infant Deaths (Apr – Dec, 2025)**	**24% (52 Infant Deaths up to Dec, 2025)**
15	Outcome	Still birth Rate	Rate	< 10 per 1000 Births	8.8
16	Output	Percentage of newborns received complete schedule of home visits against total reported live births.	Percentage	90%	**82.6%** (Against Q-3 target)
17	Output	Percentage of District Roll out HBYC visits against RoP approval with trained ASHAs	Percentage	100% (26 districts)	**88.4%** (23 districts)

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Indicator Statement	Unit	Target	Achievement
18	Output	Percentage of Districts with functional Paediatric HDU/ ICU unit out of total districts.	Percentage	100 % (12 Paediatric HDU)	92% (11 Paediatric HDUs)
19	Output	Number of facilities national certified against total identified facilities under MusQan	Percentage	100 % 16 Facilities	0 %
20	Output	Percentage of New-borns Screened at the time of Birth out of total Live Births	Percentage	100%	24%
21	Output	Percentage of DEIC functional with Infrastructure, Essential Equipment and HR as per Guidelines against the RoP approval.	Percentage	80% (5 DEICs)	60% (3 DEIC)
22	Output	Percentage of Government & Government aided schools and Anganwadi Centre covered by RBSK MHTs • Number of AWC – • Number of Govt & Govt aided schools –	Percentage	100% AWC – 8,571 School -2,290 (April to Dec 2025)	82% AWC – 6,633 (77%) School – 2,290 (100%) (April to Dec 2025)

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Indicator Statement	Unit	Target	Achievement
23	Output	Percentage of children screened by RBSK MHTs • Total number of Children in Anganwadi Centre – • Total number of Children in Government & Government aided schools –	Percentage	100% 0-6 years- 1,13,867 6-18 years- 1,91,076 (April to Dec 2025)	**78 %** 0-6 Years – 1,04,433 (92%) 6-18 Years- 1,33,288 (70%) (April to Dec 2025)
24	Output	Number of beneficiaries received Secondary/ Territory management against RoP approval (for surgical intervention specified under RBSK)	Number	40 (April to Dec 2025)	42
25	Output	Functional (including online reporting) Newborn Stabilization Units (NBSUs) against approval at CHC/FRU level	Percentage	100% (1 NBSU)	100% (1 NBSU) **Online reporting in FBNC Portal is concern**
26	Output	Health Professional (Pediatricians/ MOs/SNs/ANMs/ CHOs) Trained under Child Health Training (IMNCI/ F-IMNCI) against approval.	Percentage	100% (2 Batches)	100% (2 Batches)
27	Output	Under 5 Children received ORS and Zinc against Under 5 Children identified with Diarrhoea during the Campaign.	Percentage	100%	100%

KD's Performance (2025 – 26)

S. No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement
28	Output	Immunization	Full immunization coverage	Percentage	>=90%	91%
29	Output	Immunization	Coverage of birth dose Hepatitis B	Percentage	100%	90%
30	Output	Immunization	Drop out % of children (Pentavalent 1 to Pentavalent 3)	Percentage	0%	0.2%
31	Output	Immunization	Drop out % of children (from Pentavalent 3 to MR 1)	Percentage	0%	-5.1%
32	Output	Immunization	Drop out % of children (MR 1 to MR2)	Percentage	0%	**5.6%**
33	Output	Immunization	TD10 coverage	Percentage	>70%	**65%**
34	Output	Immunization	MR-2 coverage	Percentage	>95%	**79.5%**
35	Output	Immunization	Adherence rate of U-WIN	Percentage	Benchmark >90%	**61%**

KD's Performance (2025 – 26) (Family Planning)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target (Till Dec-25)	Achievement (Till Dec -25s)
44	Output	Family Planning	PPIUCD acceptance	Percentage	7.5%	2.53%
45	**Output**	Family Planning	Injectable MPA users	Percentage	0.3%	0.19%
46	**Output**	Family Planning	Operationalization of FPLMIS (upto the level of Sub centre)	Percentage	95%	64.77%
47	**Output**	Family Planning	% Increase in Male Sterilisation performance from 2019-20 **(40 cases)**	Percentage	200% ('2'Cases)	100% (1 case)

KD's Performance (2025 – 26) (Family Planning)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target (Till Dec - 25)	Achievement (Till Dec-25)
49	Output	Family Planning	Number of Nayi Pahel Kits (NPKs) distributed per ASHA	Number	1.5 Per ASHA (5,946 Kits)	0.37 Per ASHA (1,475 Kits)
50	**Output**	Family Planning	Number of Saas Bahu Sammelan (SBS) conducted	Number	1170	789

AH (RCH) - KD's Performance (2025 – 26) Up to Q3

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement	Low performing distt
51	Output	Adolescent Health	Average monthly Client load at AFHC/month in PE Districts at DH/SDH /CHC level	Nos	40	29	Kurung Kumey, Lower Subansiri & Namsai
52	Output		% coverage of adolescents under WIFS Programme every month.	Percentage	60%	22.74%	East Kameng, Longding, Namsai &Pakke Kessang having 0 coverage
53	Output		% coverage of out-of-school (girls) under WIFS Programme every month.	Percentage	50%	1.07%	

AH - KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement	Low performing distt
54	Output	Adolescent Health	% of Peer Educator selected against the target	Percentage	100%	24.7%	Lohit, Lower Subasani, Namsai & Changlang.
55			% of Peer Educator trained against the Peer Educator selected.	Percentage	75%	215%	
56			% coverage of Adolescent Girls against the target under Menstrual Hygiene Scheme	Percentage	NA		
57			% of the selected Districts implementing AB_SHWP against the RoP approval.	Percentage	100%	80%	
58			% of HWAs trained to transact weekly activities in schools in the select district.	Percentage	100%	**0%**	

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target (%)	Achievement (%)
1	Output	Nutrition	Early Initiation of Breastfeeding	Percentage	100	84.7
2			Bed Occupancy Rate at Nutrition Rehabilitation Centre (NRC)	Percentage		8.38 % as per state report- State has not provided any report to the division
3			Successful Discharge Rate at Nutrition Rehabilitation Centre (NRC)	Percentage		State has not provided any report to the division
4			Percentage of pregnant women given 180 IFA tablets as against pregnant women registered for ANC	Percentage	90	Chnaglang, Diband valley, east Kameng, kamle , Kra daadi and Others with 0%
5			Percentage of children 6-59 months given 8-10 doses of IFA syrup every month	Percentage	50	1.7 (all districts with 0-2%
6			Percentage of children 5-9 years given 4-5 IFA tablets every month	Percentage	30	7 (23 districts with coverage of 5% or less)

KD's for Hypertension and Diabetes (2025 – 26)

S. No.	Priority Key Deliverables (KDs)	Target for FY 2025-26	Achievement FY 2025-26 (till 17th March 2026)	%
1	% of population screened for Hypertension (Annual)	5,70,767	90,601	16%
2	% of patients on Follow-up for Hypertension	13,717	735	5%
3	% of patients under control for Hypertension	13,717	380	3%
4	% of population screened for Diabetes (Annual)	5,75,022	89,118	15%
5	% of patients on Follow-up for Diabetes	5,159	179	3%
6	% of patients under control for Diabetes	5,159	87	2%

KD's for Cancers

S. No.	Priority Key Deliverables (KDs)	Target	Achievement FY 2025-26 (till 17th March 2026)	%
1	% of population screened for Oral Cancer (FY 21-26)	5,77,940	2,11,524	**37%**
2	% of patients put on treatment for Oral Cancer against diagnosed	62	39	**63%**
3	% of population screened for Breast Cancer (FY 21-26)	2,86,080	1,03,827	**36%**
4	% of patients put on treatment for Breast cancer against diagnosed	14	2	**14%**
5	% of population screened for Cervical Cancer (FY 21-26)	2,86,080	24,124	**8%**
6	% of patients put on treatment for Cervical cancer against diagnosed	4	1	**25%**

KD's Indicators (2025 – 26)

S No.	Indicator Type	Indicator Statement	Unit	Target for FY 2025 - 26	Achievement (upto December 2025)
1	Output	Number of Districts functional under the programme	Number	23/25	**23/25**
2	Output	No. of DH with dedicated daily geriatric OPD services	Number	23/23	**23/23**
3	Output	No. of CHCs with geriatric Bi-weekly OPD services	Number	54/54	**54/54**
4	Output	No. of DH with dedicated 10 bedded geriatric wards	Number	23/23	**16/23**
5	Output	No. of DH with functional dedicated geriatric Physiotherapy Units	Number	23/23	**23/23**
6	Output	No. of CHC with Physiotherapy services for elderly patients	Number	54/54	**30/54**

As per NPHCE QPR

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement	Percentage
1	Output	NPCBVI ,NCD-1	% achievement of cataract operations against targets	Number	4500	2319	52
1	Output	NPCBVI ,NCD-1	Free Spectacle distribution to school children	Number	5450	29	1
2	Output	NPCBVI ,NCD-1	Number of donated Cornea Collected	Number	30	0	0
1	Output	NOHP	% of PHFs providing dental care services upto CHC level against total PHFs upto CHC level (DH/SDH/CHC)	Percentage	100%(114)	71 • DH: 26/26 • CHC: 35/55 • PHC/UPHC/HWC: 8/143	62%

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target 2025-26	Achievement
1	Output	National Mental Health Programme (NMHP)	Percentage of districts covered District Mental Health Units operationalized	Percentage/Number	28 Districts	16 Districts (57%)
2	Output	National Mental Health Programme (NMHP)	Percentage increase Number of persons catered through District Mental Health Units	Percentage/Number	4233(%15 increase from last year)	5,121(>100%)
1	Output	NIDDCP	% of salt samples tested using Salt Testing Kits (Qualitative testing) by ASHA in identified District. Numerator: Total Number of sample tested by ASHA. Denominator: Number of ASHA *50 samples*12months	%	100	86

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement
127	Output	NPPCD	No. of people with hearing problems rehabilitated	Number	251	301
128	Output	NPPCD	No. of people screened with deafness/hearing impairment	Number	3982	3492
1	Outcome	NTCP	Improved access for Tobacco Cessation Services Number of people availed Tobacco Cessation Services 2025-26	Number	2640	2112 (80%)

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Indicator	Unit	Target	Achievement
1	Output	NPCCHH	Orientation/ Training/Capacity Building ofhealthcare staff	% of medical officers in district trained on diagnosis and management of HRI and ARI surveillance in context of air pollution	%	80	95 **(4962/5245)**
2	Output		Heat Related Illness	% of DHs and SDH with operational min 5 bedded Heat Stroke Room (from 1st March – 31st July)	%	100	100% **(DH=30,SDH =33**
3	Output		Acute Respiratory Illness (ARI) in context of Air Pollution	% of Sentinel Surveillance Hospitals reporting daily ARI cases on IHIP portal	%	100	60%(06/10)

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement
1	Output	IDSP	% of Reporting Units Reported In 'P' form	Percentage	84%	81.48%*
2	Output	IDSP	% of Reporting Units Reported In 'L' form	Percentage	84%	81.37%*
3	Output	IDSP	% of outbreaks where preliminary assessment completed timely	Percentage	94%	48%*
4	Output	IDSP	% of outbreaks where human samples were collected	Percentage	63%	95%**

***Source of data: Period of reporting from 1st April 2025 to 24th March 2026. Data extraction from IHIP Portal as on 25th March 2026.**
****Source of data: Outbreaks which are approved and completed from 14th week 2025 to 8th week of 2026, ending on 22/02/2026.**

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Annual Target	Target till Dec	Achievement
1	Output	NVHCP	Management of Hepatitis C – under the program (% of hepatitis C patients benefitted i.e. number who received treatment against the target)	Percentage	2995 (100%)	2246 (100%)	**796** **(35%)**
2	Output	NVHCP	Management of Hepatitis B – under the program (% of hepatitis B patients benefitted i.e. number who received treatment against the target)	Percentage	1887 (100%)	1415 (100%)	**717** **(51%)**
3.	Output	NVHCP	Percentage of pregnant women screened for hepatitis B (HBsAg) against target (institutional deliveries)	Percentage	20228 (100%)	15171 (100%)	**14352** **(95%)**
4.	Output	NVHCP	Percentage of newborns administered HBIG among newborns delivered to HBsAg positive pregnant women at health care facility	Percentage	153 (100%)	115 (100%)	**14** **(12%)**

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Target	Achievement
1	Presumptive TB Examination Rate per lakh population	>2000	1076
2	Expansion of rapid molecular diagnostics for TB: % Of TB patients tested for Rifampicin resistance	70%	53%
3	State TB Score	90	80
4	Nikshay Poshan Yojana	100%	**19%**
5	Districts with TB free Status	Bronze 4	Yet to be evaluated
6	% Of Gram Panchayat/wards with TB free Status	50%	Yet to be evaluated
7	% Of consented patients adopted by Ni-kshay Mitra	100%	**29%**

KD's Performance Malaria (2025 – 26)

Sl No.	Indicator	Indicator Statement	Unit	Target	Achievement	**MoH&FW** comments
90	No. of District with API<1	Malaria Reduction in API at District level	Number	25	**25**	Achieved
91	Annual blood Examination Rate (ABER)		Percentage	10%	**10.76%**	Achieved
92	% IRS population coverage in each round		I Round	0%	0%	Only Focal IRS done.
93			II Round	0%	0%	
94	No. of Districts Certified as Malaria Free		Number	18	21 (certification under process)	Draft Dossier submitted for 16 Districts

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement
	Output	**National Rabies Control Program (NRCP)**	Availability of Rabies Vaccine till PHC level Health facilities	Percentage	**70%**	**48%**
			Availability of Rabies Immunoglobulins till CHC level Health facilities	Percentage	**40%**	**6%**
1	Dengue Case Fatality Rate (CFR- deaths per 100 cases)	Dengue & Chikungunya Division (NCVBDC)	Dengue Case Fatality Rate at State level	%	CFR<1%	**Achieved (CFR 100%)** (No death in 2025)

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement (April to till date)
1	No of Districts with Grade II Disability (G2D)% less than 2% among new cases	Central Leprosy Division	Percentage of GradeII Disability (G2D) among new cases	Number	25	25 (100 %)
2	No of Districts certified as Leprosy Free	Central Leprosy Division	Certification of Districts as Leprosy Free	Number	04	-
3	Number of Reconstructive Surgeries (RCS) conducted against back log of RCS conducted during the FY / Number of Patients Eligible for RCS during the F.Y. *100	Central Leprosy Division	Clearance of backlog of Reconstructive Surgeries (RCS)	Number	00	No RCS backlog
4	No districts with Zero new case of leprosy in the current F.Y.	Central Leprosy Division	No districts with Zero incidence of leprosy case in F.Y.	Number	-	18

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target (2025-26)	Achievement (2025-26) (Apr-Feb 26)
134	Output	Health System Strengthening (HSS)-Urban (NUHM)	Number of operational urban health facilities (UPHCs and UCHCs) increased	Percentage	UCHC – NA UPHC – 6/6	100%
135	Output	Health System Strengthening (HSS)-Urban (NUHM)	Number of UPHCs converted to Ayushman Arogya Mandir	Percentage	6/6	100%
136	Output	Health System Strengthening (HSS)-Urban (NUHM)	% of UCHC and UPHC AAMs offering specialist service.	Percentage	83%	0%
137	Output	Health System Strengthening (HSS)-Urban (NUHM)	Annual utilization of urban health facilities (UPHC-AAMs) increased with at least 50% visits made by women to be sustained	Percentage	100%	100% 51%
138 (a)	Output	Health System Strengthening (HSS)-Urban (NUHM)	Number of individuals screened for NCD at UPHC AAM: Hypertension and Diabetes	Number	100%	Hypertension – 19% Diabetes – 19%

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement
138 (b)	Output	Health System Strengthening (HSS)- Urban (NUHM)	Number of Individuals Screened for NCD at UPHC- AAM a) Oral cancer, b) Breast Cancer c) Cervical Cancer.	Percentage	100%	State is advised to focus on cancer screenings – Oral, Breast & cervical cancer
139	Output	Health System Strengthening (HSS)- Urban (NUHM)	% Urban pregnant women accessing 4 or more antenatal care at UPHC-AAM and UCHC-AAM	Percentage	80%	31%
140	Output	Health System Strengthening (HSS)- Urban (NUHM)	% of Urban Health and Nutrition Day (UHND) held organized.	Percentage	100%	29.3%
141	Output	Health System Strengthening (HSS)- Urban (NUHM)	No. of patients treated for Diabetes and Hypertension at UPHC-AAM.	Number	100%	State is advised to strengthened treatment for Diabetes & Hypertension

KD's Performance (2025 – 26)

SL No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement (till 31st January' 26)
153	Output	CP-CPHC	Percentage of functional Ayushman Arogya Mandir	Percentage	100%	92% (against approval) (511/555)
154	Output	CP-CPHC	Percentage of AAM providing 12 package of services (excluding AAM-AYUSH)	Percentage	100%	80% (409/511)
155	Output	CP-CPHC	Footfall at AAM Rural: AAM-SHC @ 300/month; AAM-PHC @1800/month - Urban: AAM-USHC @ 1200/month; AAM-UPHC @3000/month - Tribal: AAM-SHC @ 180/month; AAM-PHC @1200/month (Average population for UPHC & USHC taken from AAM Portal)	Percentage	-	AAM-SHC-3.7% (11/297) AAM-PHC-0% (1/115) AAM-USHC –0% (0/5) AAM-UPHC – 9% (06)
158	Output	CP-CPHC	% of ABHA Verified Primary Health Care Team Members registered in SASHAKT Portal/Total No. of in-position PHC team member	Percentage	100%	1.7% (79/4493)

KD's Performance (2025 – 26)

SL No.	Priority Key Deliverables (KDs)	Target for FY 2025-26	Achievement FY 2025-26 (till January 2026)						
	% of AAM CPHC team members trained on expanded service packages		ORAL	EYE	ENT	Elderly care & Palliative Care	MNS care	Emergency care	NCD
	MO (262)*		19%	24%	24%	25%	25%	22%	9%
158	SN (140)*	-	6%	7%	6%	7%	7%	7%	5%
	CHO (274)*		15%	14%	14%	16%	16%	16%	1.9%
	ANM/ MPW (901)*		3%	3%	3%	3%	3%	3%	2%
	ASHA (4076)*		12%	12%	11%	12%	12%	11%	7%

*Source- In position no from State Data and training status from SASHAKT portal

KD's Performance (2025 – 26)

SL No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement
161	Output	CP-CPHC	Percentage of facilities conducting minimum 10 wellness sessions per month	Percentage	-	**Excluding AAM AYUSH** 1.4% (6/423) **Including AAM AYUSH** 1.3% (7/511)
162	Output	CP-CPHC	Average number of Teleconsultations conducted per month per AAM (Apr to Jan 2026)	Percentage*	-	Teleconsultations per month per spoke – 1.46
163	Output	CP-CPHC	Percentage of AAM constituted JAS and conducted at least 8 meetings in last 10 months	Percentage	100%	**Excluding AAM AYUSH** JAS constituted-89% (379/423) JAS Meeting- 6.1% (26/423) **Including AAM AYUSH** JAS constituted - 81% (414/511) JAS Meeting – 4.1% (21/511)
166	Output	CP-CPHC	Percentage of AAM whose primary health care teams have received timely incentives (PBI for SHC & TBI minimum 8 times in last nine months) (Excluding AYUSH AAM)	Percentage	-	PBI-0.33% (1/297) TBI –0.23% (1/420)

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target (FY 2025-26)	Achievement
1	Output	NQAS certified public health facilities (National + State)	Percentage of NQAS Certified Facilities (a) DH (b) SDH (c) CHC (d) PHC (e) UPHC (f) AAM SC	Percentage	**50%**	DH: 5 % SDH: 0% CHC: 0% AAM-PHC: 2% AAM-UPHC: 0% AAM-SC: 8% **Total 5.21%**
2	Output	Public health facilities with Kayakalp score greater than 70%	Number of public health facilities with Kayakalp score more than 70% (on external assessment)	Nos.	**94 ***	Internal assessment of only 41 facilities completed. Peer assessment of **facilities** is on going.

*As per ROP

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target 25-26	Achievement
147	Output	Blood cell	% of DHs having functional Blood banks **(As per HDI ,State has 20 DHs. 12 DHs have BB).**	%	3 BCs at Palin, Leparada & Likabali & 1 BSU at CHC Yazali	50% (BC: 0 BSU: 2 (at Leparada & CHC Yazali)
148	Output	Blood cell	Voluntary blood donation against the blood collection units targeted for replacement/ donation	%	10392	88.58% (9206)
149	Output	Blood cell	% of blood banks having blood component separator	number	1 GH AALO	0
150	Output	Blood cell	No of integrated centers for hemoglobinopathies and hemophilia in the district against no of identified districts with high prevalence of hemoglobinopathies and hemophilia	number	1 (DH Daporijo)	1

KD's Performance (2025-26)

S. No	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement
1	Output	Free Diagnostic Service Initiative	Diagnostic Tests Availability in Aspirational blocks	Percentage	100%	33.33%
2	Output	BEMMP	Equipment CAMC/AMC	Percentage	100%	95%

Aspirational Blocks (AB) (Source: ODK toolkit)
Total AB- 03
AB having diagnostic tests availability - 01
AB not having diagnostic tests availability – 02 (Pongchau & Chowkham)

KD's Performance (2025-26)-PMNDP

Key Deliverable	Target for FY 2025-26	Achievement as on 31st Jan 2026	Remarks
Number of new HD patients registered under PMNDP(10% increase from total patients availed HD in FY 2024-25). Total patients: 254	279	112	• Operational through PPP mode • Left-out districts: 14 • Machine utilization: 1.6 • Cost per session: 1271.71 • Total sessions:12146 • No. of PD patients: 10

KD's Performance (2025 – 26)

Sl. No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement	Remarks
1	Outcome	PHA	Number of new constructions completed and handed over against the projects sanctioned.	Percentage	100% DH-2 AAM PHC-3 AAM SHC- 1 Others- 46	DH-100% (2/2) AAM PHC-100% (3/3) AAM SHC- 100% (1/1) Others- 39.13% (18/46)	• State needs to expedite the construction work and complete it within stipulated time frame.
2	Outcome	PHA	GRS & Health Help Desk: Average calls received per day: numerator- Total calls received per day per call operator against the denominator of 130	Percentage	100%	100% (Source: State Data)	• Average calls per month- 174 • Total no. of Seats- 20
3	Output	PHA	GRS & Health Help Desk : % Of calls resolved out of total calls received service wise Health Information, Counselling, SUMAN, ECD.	Percentage	100%	68%	• State needs to strengthen their Grievance Redressal Mechanism and try to resolve 100% of the calls received.

KD's Performance (2025 – 26)

Sl. No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement	Remarks
4	Output	PHA	Average number of trips per MMU per month	Number	25	18 (Source: State Data)	• The average no. of trips each MMU conducts is 18 per month
5	Output	PHA	Average no. of lab investigations per MMU per day	Numbers	40 in Hilly	Hilly- 35 (Source: State Data)	• On an average, each MMU conducts 35 tests per day
6	Output	PHA	National Ambulance Services : % of Ambulances functional as per population norms (one BLS per 1 lakh Population and One ALS for every 5-lakh population) National Ambulance Services : Average response time per vehicle	Percentage Number	100% 20 Min	100% BLS= 110 ALS= 0% (4 reqd) Avg response time per vehicle: Rural- 16.08 mins • Urban- 14.30 mins	• Total No. of Ambulances in state: ALS= 0 BLS= 110 • State to saturate ALS as per norms. • Average ambulance response time is 15.19 minutes.

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement	Remarks
7	Process	PHA	% of District hospitals-initiated any of the following courses:- DNB courses Nursing courses Allied health care courses	Percentage	NA	NA	• State is suggested to start DNB courses in all the DHs as part of DH strengthening

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement	Remarks
1	Outcome	PHA	IPHS Compliance - %age of healthcare facilities achieved IPHS Compliance.	Percentage	50%	**2%** (10/609) DH: 1/22 SDH: 0/0 CHC: 0/56 AAM-PHC: 0/131 AAM-HSC: 9/400 (Source: IPHS Dashboard 20/03/2026)	• State to achieve the NHM conditionality target of 50% IPHS compliance. • State to prioritize facilities currently falling under progressive category (Scoring70–79% 50-69%) to achieve full IPHS compliance (≥80%) of 39% of all facilities. • Upgrade all CHCs to FRUs and strengthen FRUs to ensure a continuum of care across all 12 essential services.
2	Output	PHA	National Ambulance Service – Average No. of Patients Transferred Per Month Per Ambulance	Number	50 patient/per ambulance/per month	Avg **9.23** patient/per ambulance/per month (Source: State Report as on Dec.25)	• As per population norms State requires 4 ALS & 20 BLS. State to saturate ALS as per norms. State has achieved BLS saturation with 140 BLS. State also has 149 PTVs. • Establish command and dispatch centre with 112 for end to end emergency response. • Conduct quarterly ambulance quality audits with corrective-action closure. • Ensure 100% GPS enablement and AIS-125 compliance.

KD's Performance (2025 – 26)

Sl No.	Indicator Type	Division	Indicator Statement	Unit	Target	Achievement	Remarks
3	Output	PHA	MMU – Average No. of Camps held per MMU and average Footfall per Camp	Numbers	Av. camps held per MMU/month- 25 camps Average footfall/day- 50 patient	Av. camps held per MMU /month- **18** Average footfall/day -**80** (Source: State Data)	• State has 16 MMUs (**as per NHM MIS**). • Ensure timely entry and regular updates on the MMU portal. • Rationalize routes to cover remote habitations; implement fixed-day micro-plans. • Deliver the full 12-service package with referral linkages and close monitoring/supervision.

CRM ACTION TAKEN REPORT

Arunachal Pradesh was covered in 16th CRM

CRM Action Taken Report

SL.	Observations of CRM Team	Action taken by the states	Remarks of division
1	Out of the 12 CPHC service packages, an average of 7 packages were available in West Siang District and 9 in Longding District. Key gaps included lack of training for CHOs in eye care, ENT, palliative care, geriatrics, mental health, cervical cancer screening (VIA), and IUCD insertion.	CHOs have been reoriented during routine district and state-level review meetings. Training on expanded package except cervical screening (VIA) done for CHO/HWO/Nursing Officers/ANM. Identified cases are being aligned to referred to higher facilities as per protocol.	The State may develop a focused training plan to address gaps including cervical cancer screening (VIA). Strengthening supportive supervision, on-site mentoring and ensuring necessary logistics and referral linkages will help improve service delivery. State to propose for Integrated Module Training for CHOs in PIP 2026-27.
2	Community awareness of AAM services was limited, with most visits centred around routine medication refills for diabetes and hypertension. Awareness of wellness activities organized at AAM facilities was notably low.	Wellness activities are being conducted at AAM facilities and monthly Ayushman Arogya Shivirs are conducted on fixed days. IEC/BCC activities intensified through VHSNDs, ASHA and PRI involvement (JAS) to enhance service utilization.	Awareness activities may be organized on calendar days at the community level. AAMs can collaborate with departments like Social Welfare, PHED, and Education to promote services in sectoral meetings, with support from VHSNC and JAS members.

CRM Action Taken Report

SL.	Observations of CRM Team	Action taken by the states	Remarks of division
3	AAM facilities in West Siang had adequate waiting areas, dedicated spaces for labs, labour rooms, and wellness activities, and sufficient HRH as per CPHC guidelines and IPHS. In contrast, Longding faced infrastructural challenges, including limited space, absence of separate toilets, lack of piped water supply, staffing shortages, poor approach roads, intermittent power supply, and weak internet connectivity.	Services are being continued with available infrastructure and temporary arrangements. Proposals for minor civil works, water supply, power backup and internet connectivity to be submitted under NHM/state plan.	The State may closely monitor the progress of proposed infrastructure improvements and ensure timely completion. A system for periodic review and certification of completed works may be instituted to ensure quality and functionality of facilities.
4	Branding of AAM facilities was implemented in West Siang but not in Longding. The state had not uniformly rolled out AAM branding across districts.	Districts have been sensitized to strictly adhere to the prescribed branding guidelines. The PHED & WS has now been engaged as the executing agency for all health infrastructure–related activities, including AAM branding, to ensure uniformity, quality compliance, and smooth implementation across the State.	The State may ensure time-bound and uniform implementation of AAM branding across all districts with regular monitoring.

CRM Action Taken Report

SL.	Observations of CRM Team	Action taken by the states	Remarks of division
5	Inadequate availability of essential drugs and diagnostics with frequent stock-outs	Regular indenting and emergency procurement are being undertaken at the district level. Further strengthening of supply chain management is being ensured through effective utilization of the DVDMS platform.	The State should ensure availability of medicines as per EDL and take measures to prevent frequent stock-outs through strengthened inventory management, timely replenishment, and regular monitoring.
6	Limited availability of diagnostic tests at AAM-SHCs compared to PHCs	Available diagnostics are being optimally utilized. Expansion of diagnostic services at AAM-SHCs is proposed in alignment with CPHC norms under the Free Drugs and Diagnostics Initiative.	The State may ensure expansion of diagnostic services at AAM-SHCs in a phased manner by prioritizing high-load facilities first, followed by gradual scale-up across all centres, with regular monitoring of availability and utilization.

CRM Action taken report

SL.	Observations of CRM Team	Action taken by the states	Remarks of division
7	Low utilization of teleconsultation services due to poor network connectivity.	Teleconsultation services initiated wherever feasible. Strengthening of teleconsultation services in low bandwidth is being explored.	State may also explore the provision of Bharat net or any other available services
8	Cancer screening services remained inconsistent, with cervical cancer screening (VIA) unavailable at the primary level due to community reluctance.	The State has initiated measures to strengthen cervical cancer screening by planning capacity building of nurses/ANMs in VIA, along with intensified IEC and community counselling through ASHAs and Ayushman Arogya Mandirs to address social reluctance; phased rollout of VIA services at selected primary health facilities is being undertaken.	The post implementation impact following the initiative taken by the state may be reviewed.
9	While eligible ASHAs were enrolled under PMJJBY and PMSBY, there is no provision for death compensation. Many ASHAs were unaware of social security provisions and lacked clarity on financial incentives.	ASHAs have been oriented on social security scheme and its benefits.	A state level review may also be done regarding the claim settlement percentages and or any issues being faced by ASHAs in claiming the benefits.

CRM Action taken report

Sl.	Observations of CRM Team	Action taken by the states	Remarks of division
10	The ASHA selection process is conducted by the Village Council at the community level, ensuring local involvement. ASHAs were motivated, active, and well-informed about their community-level responsibilities. They were trained in HBNC, HBYC, NCD, maternal and child health, but need training on the Expanded Package of Services (EPS). Also, regular capacity building programme may be conducted for ASHAs to update them regarding new programmes and interventions.	Training for ASHA under EPS has been proposed under CPHC for FY 2026-27. State TOT for the district trainer will be organised at the state level. Further the district trainer will train the ASHAs at the district/block level. The training fund will be decentralised to the district. (CPHC-EPS).	The suggested number of training days may be followed as per the norms of Integrated module training.
11	The JAS have been formed at both AAM-SHCs and AAM-PHCs. While AAM-PHC JAS registers were well-maintained and meetings were held every one to two months, AAM-SHC JAS meetings was not regular.	Instructions from the state communicated to the districts for conducting regular meetings. (CPHC)	JAS re-orientation may also be given to the JAS members on record keeping.

16th CRM Action taken report

S. No.	Observations of CRM Team	Action taken by the State	Remarks of division
1	Despite these efforts, critical gaps in the availability of essential medical supplies were observed. Both the anti-rabies vaccine and rabies immunoglobulin were either unavailable or out of stock at the CHC and DH in the West Siang district. Addressing these shortages is essential to ensuring timely and effective rabies treatment.	1. Supply chain gaps have been identified, and stock monitoring mechanisms are being reviewed. 2. Ensuring ARV and RiG availability is being worked on to search for extra funds 3. Districts have been advised to report stock levels regularly 3. The State has initiated corrective actions to strengthen NRCP logistics and procurement systems.	• State needs to ensure the availability of medicines as per at all levels of public health facilities. • State should regularly monitor stock through DVDMS and establish a monitoring system for the availability of medicines at facility, block, district, divisional, and state levels. • As per DVDMS central dashboard stock availability as on 24th March 2026 at DH (6.16%), CHC (5.47%), AAM-PHC (6.23 %), AAM-SHC (4.23%).
2	It is essential to strengthen bidirectional referral of stable NCD cases and ensure the availability of common NCD drugs at the primary care level.	The State is strengthening bidirectional referral and back-referral mechanisms between AAM, PHC, CHC and District Hospitals to enable management of stable NCD cases at the primary level; efforts are being made to ensure uninterrupted availability of essential NCD drugs at primary health facilities through NHM and state procurement systems.	

16th CRM Action taken report

S. No.	Observations of CRM Team	Action taken by the State	Remarks of division
3	Frequent stockouts of essential medicines (especially for hypertension and diabetes) were observed, with 15-day lead times for replenishment	Arunachal Pradesh is strengthening forecasting and procurement through centralized indenting, buffer stock maintenance at district level, and closer monitoring of drug availability through DVDMS. Districts have been directed to prioritize uninterrupted supply of essential NCD medicines.	• Drug forecasting methodologies (e.g., VEN, FSN, HML) need to be further standardized across all levels of facilities to ensure the availability, accuracy, and uniformity of medicines. • Regular monitoring of EDL medicines at facility, block, division, district, and state levels is also recommended.
4	Inadequate availability of essential drugs and diagnostics with frequent stock-outs	Regular indenting and emergency procurement are being undertaken at the district level. Further strengthening of supply chain management is being ensured through effective utilization of the DVDMS platform.	

CRM FINDINGS

Observation	State Action Taken	Remarks of division
• Monthly water endotoxin checks, and quarterly viral marker assessments were not conducted as required • The remaining 16 districts had no dialysis centers and were linked with the existing functional units	1. Routine dialysis services are ongoing; basic infection control practices are in place. 2. SOPs to be reinforced with PPP partners and timely Monitoring using NQAS standards 3. Mandatory monthly water endotoxin testing and quarterly viral marker testing to be ensured through contractual clauses and regular audits. 4. Increasing the Ex. cost per Dialysis from Rs 1400 to Rs 2500 to ensure single usage of tubings to ensure avoidance of chance lapse of quality by Technicians 1. Referral linkages established with existing dialysis centres for uncovered districts. 2. Hub-and-spoke model operationalised; 3. CKD screening to be strengthened under NP-NCD to reduce late referrals	State needs to monitor and strengthen quality of care in dialysis unit. State is advised for NQAS certification.

CRM FINDINGS

Observation	State Action Taken	Remarks of division
• The services were operated on a PPP model in the state. The PMNDP portal usage was around 84%. • In West Siang District (DH Aalo) the HD center had 03 machines while these were missing in Longding district.	1. Dialysis services are being provided through PPP mode at designated facilities 2. PMNDP portal is being used for patient registration and service reporting. 3. Portal usage to be increased to 100% by ensuring ABHA ID generation for all dialysis patients 4. Regular quarterly review meetings to be conducted at State and District level to monitor PMNDP portal reporting and data quality. 1. HD services are available at DH Aalo; 2. Patients from Longding district are being referred to functional dialysis centre at Khonsa in a Hub and Spoke model due to low patient load. 3. Interim strengthening of referral transport support for patients being worked on	PMNDP programme is running satisfactorily in the state.

CRM FINDINGS

Observation	State Action Taken	Remarks of division
• The state had an MoU with Sir Ganga Ram Hospital, New Delhi for review of dialysis patients, training and assistance in renal transplant which is a novel concept	1. MoU signed and operational for expert review, training support and guidance on renal transplant care. 2. Nephrology Unit established at TRIHMS 3. Expansion of tele-nephrology consultations; capacity building of state clinicians; development of long-term roadmap for renal transplant referral and follow-up care.	State needs to monitor the Nephrologist consultation/visits at the dialysis centres.
• User Fees levied for Diagnostics and Lab tests at DH & CHC level • DH has two Labs- one operated by PPP and other in house	1. State Specific Essential Diagnostic list for each level of Health Facility is approved 2. LMIS is proposed to be introduced to ensure robust supply chain and logistics management 1. The high volume low costs tests to be performed in house and low volume high cost tests to be outsourced. The advanced tests that are low volume, high cost tests to be performed at Hub and Spoke Model in PPP Mode.	State needs to strengthen the diagnostic capacity & availability of tests at all level of health facilities.

CRM FINDINGS

Observation	State Action Taken	Remarks of division
• X-Ray Machines found non functional and patients were referred to private clinics.	1. XVFC Health grants funds is to be utilized for AERB Compliance of the DH and CHC and also to procure X-Ray machines with CR/DR System where required. Reporting to be done through Tele – Radiology.	State needs to strengthen the AERB compliance.

CRM Action Taken Report

Sl. No.	Observations of CRM Team	Action Taken by State	State Remarks	**Remarks of division**
NCD 11 NPPCD 1	Challenges in service expansion due to limited availability of specialized workforce (ophthalmic, ENT, dental, and mental health professionals), inadequate equipment and consumables, and weak referral coordination for advanced care, resulting in delays in treatment initiation.	Arunachal Pradesh faces similar challenges due to difficult terrain and limited availability of specialists at district and sub-district levels. The State is addressing this through task-shifting, specialist outreach camps, tele-consultation services, prioritized procurement of essential equipment, and strengthening referral linkages with TRIHMS and other tertiary centres.	Gaps persist in remote areas; phased strengthening through telemedicine, HR rationalization, and improved referral mechanisms is ongoing.	Noted. State to fill vacancies under regular cadre to ensure availability of human resources and improve service provision.

CRM Action Taken Report

Sl. No.	Observations of CRM Team	Action Taken by State	Remarks of division
RCH 1 MH 5	Shortage of MBBS Medical Officers; PHC-AAMs in Longding largely managed by AYUSH MOs	Engagement of MBBS graduates from medical college is being undertaken for such PHC-AAMs. Telemedicine support to AYUSH MOs is being strengthened through infrastructure augmentation and linkage with MBBS MOs and specialists at designated hub facilities.	Noted
RCH 1 MH 9	NRC at Longding was not functional due to HR shortages.	Proposal for recruitment postponed for FY 2026-27 due to problem in accommodation with RE of FY 2025-26 PIP	Noted. State to rationally deploy existing staff as per IPHS norm and caseload and not as per vertical programmes.

CRM Action Taken Report

Sl. No.	Observations of CRM Team	Action Taken by State	Remarks	Remarks of division
RCH 1 MH 12	Delay in implementation of key trainings (SBA, NSSK, KMC, IMNCI, FIMNCI, FBNC, MDR, CDR)	Time-bound training calendar with monitoring and evaluation to be implemented	Arunachal Pradesh has made efforts to implement various key trainings, including SBA, NSSK, KMC, IMNCI, FIMNCI, FBNC, MDR, and CDR, under the National Health Mission (NHM). The State is appointing a dedicated State Nodal Officer (Training) to focus on the following: 1. Strengthening the training infrastructure and capacity 2. Regular monitoring and evaluation of training programs 3. Ensuring availability of trained healthcare providers at facilities Improving community awareness and demand for services	Noted. State to ensure officer from the regular cadre to be designated as the Nodal Officer. State should prepare a comprehensive training calendar as per RoP approvals to streamline the management of capacity building initiatives.

Annexures

Capacity building - 201